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Clinical Diagnosis & History:

History of right colon cancer and diverticulitis. Here for total abdominal colectomy.

Specimens Submitted:

1: SP: Terminal ileum and colon; total abdominal colectomy

DIAGNOSIS:

1. SP: Terminal ileum and colon; total abdominal colectomy:

Tumor Type:

Mucinous adenocarcinoma

with a minor component that is of conventional (gland-forming)

type

Histologic Grade:

Moderately to poorly differentiated

Tumor Location:

Ascending colon

Tumor Size:

Length is 3.0 cm

Width is 2.6 cm

Maximal thickness is 0.9 cm

Tumor Budding:

Extensive

Increased Tumor Infiltrating Lymphocytes:

Present

Precursor Lesions:

Tubulovillous adenoma

Deepest Tumor Invasion:

Subserosal adipose tissue and/or mesenteric fat

Gross Tumor Perforation:

Not identified

Lymphovascular Invasion:

Identified

Large Venous Invasion:

Suspected

Perineural Invasion:

Identified

Surgical Margins:

Free of tumor

Polyps/Mucosa Dysplasia (away from the carcinoma):

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Tubular adenoma; number: 2
Non-Neoplastic Bowel:
Diverticulosis
of left colon
Appendix:
Unremarkable
Lymph Nodes:
Number with metastasis: 1
Total number examined: 46
Tumor deposits in pericorectal soft tissue:
Not Identified
Tumor Staging (AJCC 7th Edition):
pT3 (Tumor invades through the muscularis propria into
pericorectal tissues)
Lymph Node Stage (AJCC 7th Edition):
N1 (Metastasis in 1-3 regional lymph nodes)

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF
THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED
THIS REPORT.

Gross Description:

1. The specimen is received fresh, labeled "total abdominal colectomy" and consists of a segment of terminal ileum, cecum with attached appendix and colon. The terminal ileum measures 11.0 cm in length and 4.2 cm in circumference at the proximal resected margin. The remaining colon measures 96.0 cm in length with a circumference of 6.0 cm at the distal resected margin. The attached appendix measures 8.0 cm in length and averages 0.7 cm in diameter. The appendiceal and intestinal serosa is pink tan and smooth. Focally hemorrhagic lobulated yellow tan adipose tissue spans the length of the specimen measuring up to 1.3 cm in thickness. The specimen is opened to reveal a mass lesion measuring 3.0 cm in length and 2.6 cm in width. The mass is located in the ascending colon, 16.1 cm from the proximal margin and 89.0 cm from the distal margin. Sectioning shows that the tumor invades into the muscularis propria. The depth of invasion is 0.9 cm grossly. There are two polyps measuring 0.2 and 0.7 cm in the transverse colon. Diverticula and muscle wall thickening are present in the distal 14.0 cm of the colon. The remaining mucosa is grossly unremarkable. The attached adipose tissue is thoroughly examined and all identified lymph nodes are submitted. Representative sections of the specimen are submitted for permanent sections and for TPS.

Summary of sections:
P - proximal margin shave

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D - distal margin shave
T - tumor
POL- two polyps in transverse colon
DIV- diverticula
A - appendix representative sections
RS -representative sections
LN - lymph nodes
BLN - bisected lymph nodes

Summary of Sections:

Part 1: SP: Terminal ileum and colon; total abdominal colectomy

Block	Sect. Site	PCs
1	a	1
1	bln	1
1	d	1
4	div	4
10	ln	25
1	p	1
1	pol	1
1	rs	1
4	t	4

Addendum Diagnosis

-RESULTS OF IMMUNOHISTOCHEMICAL STAINING FOR DNA MISMATCH REPAIR PROTEINS
ARE AS FOLLOWS:

MLH1: STAINING PRESENT IN TUMOR

MSH2: STAINING ABSENT IN TUMOR

MSH6:STAINING ABSENT IN TUMOR

PMS2: STAINING PRESENT IN TUMOR

CONCLUSION:

-IMMUNOHISTOCHEMICAL STUDIES DEMONSTRATE ABNORMAL MSH2 AND MSH6 PROTEIN
EXPRESSION IN THE CARCINOMA. CLINICAL CORRELATION IS SUGGESTED.

NOTE: STAINS PERFORMED ON A SEPARATE ADENOMA [REDACTED] SHOW A NORMAL
STAINING PATTERN FOR ALL 4 PROTEINS (MLH1, MSH2, MSH6 AND PMS2).

Some of the immunohistochemistry and ISH tests were developed and their
performance characteristics were determined by the Department of Pathology.
They have not been cleared or approved by the US Food and Drug
Administration. The FDA has determined that such clearance or approval is

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not necessary. These tests are used for clinical purposes. They should not
be regarded as investigational or for research. This laboratory is certified
under the Clinical Laboratory Improvement Amendments of [REDACTED] as
qualified to perform high complexity clinical laboratory testing.

[REDACTED]

** End of Report **